Supplementary material 2. Codebook of the qualitative analysis. The table presents the final analytical structure emerging from the qualitative analysis, including the five categories, their associated codes and sub-codes, operational definitions, and the number of interviews and coded segments supporting each code.

Category/code	Definition	Interviews	Segments
Category 1 – Social representations and relationship to stereotypes of chronic pain	Representational frameworks concerning chronic pain and people living with pain, including bodily, behavioural and moral stereotypes, intra- and intergroup judgements, and typified prototypes. These representations structure the social evaluation of pain and shape interactions. The category also includes the processes through which people actively position themselves in relation to these representations, by contesting, transforming or internalising them on the basis of their lived experience.	30	234
Socio-cognitive positioning in relation to pain representations	Processes through which participants actively position themselves in relation to social representations of chronic pain, by contesting them, distancing themselves from them, or transforming them on the basis of their lived experience.	24	66
Distancing from stereotypes of chronic pain	Process through which participants explicitly or implicitly contest homogenising or devaluing stereotypes associated with people living with chronic pain, by distancing themselves from these images or refusing their application to themselves or to their group.	15	26
Integration of medical models of pain	Process through which people incorporate, over the course of their interactions with healthcare professionals and care systems, biomedical or biopsychosocial explanatory frameworks of chronic pain, such as brain mechanisms, chronification, or the distinction between acute and chronic signals. These models become interpretive resources mobilised in their own account to make sense of their experience, independently of their translation into behavioural strategies.	9	10
Transformation of representations through the experience of pain	Process through which participants describe an explicit change in their previous representations, concerning pain, people living with pain, or themselves, attributed to their own lived experience of chronic pain.	16	30
Intergroup representations of chronic pain	Representations that participants attribute to people who do not live with chronic pain, including healthcare professionals, relatives, colleagues, or society, concerning chronic pain and people who suffer from it. Whether they take the form of minimisation, moral suspicion, such as malingering or laziness, or psychologising reduction, these representations constitute an interpretive frame through which pain is evaluated, judged, and made more or less legitimate.	21	55
Social categorisation through chronic pain	Social representation of a classificatory practice through which chronic pain is perceived as a criterion of identity assignment, an explicit or implicit placing into a category by the exogroup, including society, healthcare professionals, or close others. Participants describe this categorisation as reductive and potentially stigmatising, particularly in the case of invisible disabilities, where the legitimacy of occupying the category of patient is contested.	8	12
Intragroup representations of people living with chronic pain	Representations and norms developed by people living with chronic pain about other people living with chronic pain. This includes typified representations of the person in pain, or prototypes, normative judgements about legitimate or inadequate ways of living with pain, and causal or dispositional representations attributed to the endogroup.	28	113

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Category/code	Definition	Interviews	Segments
Internalisation of the naturalisation of pain	Process through which participants themselves interpret their own chronic pain as a normal, expected, or inevitable phenomenon, particularly in relation to age, ageing, or bodily wear. In contrast to intergroup representations of naturalisation, this code captures the person's adherence to this interpretive frame within their own account.	13	28
Intragroup normative judgements	Evaluative judgements made by people living with chronic pain about other people living with pain, based on implicit norms of acceptable ways of managing pain, such as self-control, restraint in complaint, maintenance of appearance, therapeutic effort, and individual responsibility. These judgements distinguish behaviours viewed as legitimate, such as continuing to act, not complaining excessively, and maintaining self-presentation, from behaviours perceived as excessive, resigned, or inadequate.	14	35
Prototype of the person living with chronic pain	Elaboration of a typified representation of the person living with chronic pain, combining perceived bodily signs, such as grimacing or neglected appearance, observable behaviours, such as social withdrawal or complaint, and assumed psychological dispositions, sometimes ambivalent, such as resignation or irritability. This prototype may be elaborated from the endogroup position or internalised from the gaze attributed to the exogroup, and constitutes a perceptual script that guides the social evaluation of pain.	20	49
Category 2 – Relational and institutional dynamics of chronic pain	Mechanisms through which the social environment, from immediate interactions to organisational structures, shapes the recognition, legitimacy, and management of chronic pain. Whether these mechanisms take the form of interactional stereotypes, relational asymmetries, normative or institutional constraints, or conversely supportive resources, they threaten or preserve the needs for autonomy, belonging, and recognition, and condition agency.	30	703
Asymmetry of experiential understanding	Structural limit of understanding between the person in pain and their interlocutors, taking three forms: an epistemological gap based on the absence of an analogous embodied experience; normalisation by analogy with ordinary or transient pain; and the maintenance of unchanged expectations despite declared knowledge of the situation. This differs from active delegitimation: asymmetry may exist without an intention to reject.	24	85
Normalising comparison and empathic recentering	Interactional mode in which the interlocutor responds to expressed pain by comparing it with their own experience of ordinary pain, or by recentring the account on themselves, producing an implicit banalisation of the specificity of chronic pain.	6	10
Decision-making asymmetry and relational paternalism	Situations in which a professional, institution, or close other adopts a decision-making or prescriptive position on behalf of the person in pain, reducing their capacity to interpret and manage their own situation. The asymmetry may be institutional, linked to expert authority, or affective, linked to protection under the guise of care, and manifests through injunctions, unilateral decisions, or imposed advice, regardless of the third party's intention.	18	46
Delegitimation of patients' accounts	Situations in which the person's description of their pain is actively called into question through psychologisation, suspicion of malingering or exaggeration, explicit rejection, or non-verbal signs of non-recognition. This differs from asymmetry of understanding by its active and intentional character. This mechanism challenges the reality, severity, or validity of lived experience.	24	70

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Category/code	Definition	Interviews	Segments
Interactional effects of stereotypes	Situations in which pre-existing stereotypes about what a person in pain should look like or how they should behave automatically guide the interpretation of observable behaviour and structure the interaction accordingly, whether through suspicion of malingering, attribution of excessive assumed capacity, or psychologisation of lived experience. These interpretive biases, activated without the interlocutor's awareness, produce inadequate responses that threaten recognition of pain and the sense of justice.	24	80
Avoidance, weariness, and social rejection	Behaviours from the social environment marked by avoidance, weariness, active rejection, or indifference towards the person in pain, leading to a reduction or rupture of social ties. Whether motivated by discomfort in the face of suffering, helplessness, or hostility, these behaviours directly threaten the need for belonging and may amplify the perception of social pain.	13	24
Organisational logics and rationalisation of care	Mechanisms through which organisational features of healthcare shape patients' experiences of recognition, control and agency in the management of chronic pain and the healthcare professional-patient relationship. These constraints, whether related to access and waiting times, fragmentation of the pathway, rationalisation of acts, or administrative barriers, limit the person's agency over their therapeutic trajectory and may, in interaction with stereotypes, produce inequalities in treatment.	27	153
Access and temporality of care	Constraints related to waiting times, limited availability of professionals, territorial or administrative rules of the healthcare system, and institutional barriers to recognition of rights, such as disability services, expert assessments, or insurance procedures, which delay or limit access to appropriate care. These constraints, external to the person, reduce their power to act on their own therapeutic trajectory and limit their agency.	19	42
Fragmentation and discontinuity of the pathway	Absence or weakness of coordination between professionals, interruptions in therapeutic protocols, and contradictions between recommendations, producing a fragmented and unstable care pathway. These discontinuities, passively endured, generate a loss of control over the therapeutic trajectory, decision-making overload, and limit agency by reducing the continuity of care resources.	16	48
Prescriptive logic and medical rationalisation	Situations in which medical care is structured around standardised procedures, such as prescriptions, examinations, protocols, or measurement tools, at the expense of listening time and adjustment to the singular lived experience of chronic pain. This rationalisation deprives the person of recognition and validation of their experience, and may lead to an agentic impasse when available therapeutic resources are declared exhausted.	22	63
Normativity and constraints of the professional world	Mechanisms through which performance norms and institutional rules in the world of work constrain the expression and management of chronic pain. These constraints operate at two levels: acts of disqualification and discrimination in the immediate professional environment, and structural failures of workplace adjustment and job retention systems.	21	74
Insufficient adjustments and structural discrimination	Situations in which formal workplace adjustments, such as therapeutic part-time work, disability recognition, or job adaptation, prove inadequate, are not applied, or are circumvented in practice, or where institutions ignore the person's documented limitations. These repeated experiences of powerlessness over the professional trajectory may produce progressive exclusion from work, threatening economic autonomy and the sense of social worth.	15	35

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Category/code	Definition	Interviews	Segments
Disqualification and discrimination in the immediate professional environment	Attitudes, comments, decisions, or deliberate disregard of known limitations by colleagues, supervisors, or human resources services, which minimise pain, question the legitimacy of sick leave or status, or make unfavourable decisions against the person in pain. Whether explicit or silent, these acts threaten the need for recognition and justice in the immediate professional space and may lead to constrained exits from employment.	13	39
Normative pressures of performance and normality	Situations in which the person in pain is confronted with explicit or implicit expectations to conform to norms of performance, productivity, appearance of good health, or maintenance of social roles, through direct injunctions, unsolicited advice, or social conventions, including cultural norms about the legitimacy of complaint, which encourage them to hide or minimise their pain. Failure to comply with these norms produces comments, doubts, or losses of social legitimacy.	18	49
Social support	Relational resources mobilised to accompany the person in the experience of chronic pain, whether from family, friends, colleagues, peers, or healthcare professionals. This support is structurally ambivalent: protective when it explicitly recognises pain and adjusts to limitations, but fragile over time, conditional in its forms, and liable to shift towards paternalism or indifference.	24	122
Temporal erosion of support and social adjustment	Process through which support and behavioural adjustments by the social environment, initially present, gradually decrease over time through habituation to the signal of chronic pain, while the person's limitations persist. This progressive extinction of the empathic response, experienced as forgetting or implicit normalisation, threatens the need for belonging and may reinforce the self-silencing of pain.	10	16
Social support at work	Practical help, informal adjustments, or benevolent attitudes from colleagues or managers that explicitly recognise pain-related limitations and adjust their expectations or behaviours accordingly. When adjusted and non-intrusive, this support validates the legitimacy of pain in the professional space and preserves social participation and the sense of belonging tothe work collective.	6	11
Social support from close others	Relational resources mobilised by family, friends, orthe personal environment to accompanythe person in pain: practicaland logistical help, emotional validationof lived experience, suggestionsof activitiesand escape, or support through shared vicarious experience. When adjustedand explicitly recognisingthe realityof pain, this support preservesthe senseof belongingand everyday agency. It may be ambivalent: benevolent in intention but conditional over time.	20	56
Social support from healthcare professionals	Attitudes and practices of healthcare professionals that go beyond the technical act to explicitly recognise pain, attend to the person as a whole, and maintain an adjusted human relationship. When present, this support constitutes an experience of direct recognition that contrasts with delegitimising interactions and strengthens the sense that the situation can be accompanied, even if not resolved.	12	39
Category 3 – Threat to the social self and to agency	Processes through which chronic pain weakens the social self: internalisation of others' gaze, incorporation of pain into identity, perception of oneself as a burden, contraction of the relational world, and threat to fundamental needs for autonomy, belonging, and recognition. These processes, endured and not deliberate, modify the person's perceived place, legitimacy, and sense of control over their own trajectory.	30	512

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Category/code	Definition	Interviews	Segments
Alteration of self-image under the social gaze	Internalisation of social threat through which the person living with chronic pain weakens their self-image by integrating the real or anticipated gaze of others. This process, endured and not deliberate, affects the sense of worth, usefulness, or legitimacy through continuous cognitive monitoring of the self as a social object, threatening the need for recognition.	25	107
Pain as part of the self	Process through which chronic pain is incorporated into personal identity. It is no longer perceived only as a symptom, but as a lasting component of the self, through habituation, acceptance, or transformation, affecting representations of the self over time, comparison with the former self, projections into the future, and relationships with others. This process may be integrative or invasive depending on whether it preserves or constrains agency.	11	15
Perception of oneself as a social burden	Process through which the person living with chronic pain comes to perceive themselves as a burden on their social environment or on society. This perception, often based on an anticipatory inference of others' judgement rather than explicit reproaches, is accompanied by a sense of loss of usefulness and social worth, limiting agency and weakening the sense of social legitimacy.	17	36
Contraction of the social world	Process through which chronic pain and its constraints progressively reduce activities, interactions, and social commitments, under the effect of cognitive and emotional exhaustion and anticipatory avoidance of situations of judgement or implicit exclusion. The result is isolation that is both endured and linked to experiential avoidance, with a narrowing of the relational world around a restricted core. This narrowing weakens the sense of belonging and limits the capacity to act.	26	77
Threat to fundamental social needs	Parent code organising the three dimensions through which chronic pain threatens fundamental social needs: autonomy, belonging, and justice or recognition, producing a perceived imbalance in the person's place, legitimacy, and power to act within their social environment.	30	328
Threat to justice and social recognition	Experiences in which chronic pain is associated with a sense of injustice or non-recognition, whether procedural, such as not being believed, heard, or treated fairly in interactions, or outcome-related, such as perceiving one's situation as unfairly treated in view of the magnitude and irreparability of the loss endured. This sense operates at relational, institutional, and normative levels, and is modulated by gender and ethnic origin. It weakens the sense of legitimacy.	25	135
Threat to the sense of belonging	Experiences in which chronic pain weakens the sense of belonging to a meaningful group, by producing a cognitive and perceptual gap between the person and close others, colleagues, peers, or society, that is, experiential asymmetry. What matters is the subjective perception of exclusion or peripheral position, regardless of objective isolation. This also covers the ambivalence of peer groups of people in pain: mutual recognition versus enclosure within an illness identity.	24	84
Threat to autonomy	Experiences in which chronic pain produces a sense of loss of control over one's own actions and life trajectory, through the constrained displacement of agency towards others, including close others, institutions, and the healthcare system. This sense, endured and not deliberate, differs from physical functional limitation by focusing on the experience of identity dispossession: no longer being able to decide, move around, provide for oneself, invest in the intimate sphere, or act by oneself.	24	109

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Category/code	Definition	Interviews	Segments
Category 4 – Deliberate strategies for restoring agency	Deliberate behaviours through which the person restores or preserves their sense of control, self-determination, and self-efficacy in the face of constraints imposed by chronic pain and the social environment. These strategies act on the body, therapeutic decisions, social commitments, and management of information about pain, in response to threats to the needs for autonomy, belonging, and recognition.	30	435
Interoceptive monitoring oriented towards functional control	Deliberate process through which the person maintains active surveillance of bodily sensations and pain signals in order to anticipate worsening, calibrate effort, and adjust activities in real time. This interoceptive monitoring, oriented towards a functional goal, enables the person to preserve a sense of control over the body and social participation despite the unpredictability of pain, constituting a form of interoceptive expertise built over the course of the pain experience.	15	37
Active reconstruction of agency	Deliberate steps through which the person maintains or regains their power to act on their body, therapeutic choices, and social relationships, in response to threats to autonomy produced by chronic pain and the social environment. This parent code documents the recovery of control over the care pathway and the deliberate physical effort of regaining control over the body. The subcodes document its relational, identity-related, and assertive dimensions in the face of assignments.	30	219
Selective affiliation and restoration of agency	Deliberate redirection of relational investments towards social contexts perceived as understanding or valuing. This takes two complementary forms: affiliation with peers sharing a similar experience, and engagement in prosocial roles of helping or transmitting experience. These strategies produce experiences of competence, legitimacy, and belonging that restore agency in the face of the social threats of chronic pain.	29	90
Peer relational resources (endogroup)	Deliberately sought interactions with peers living with similar pain, which reduce the cognitive burden of legitimation, provide emotional validation and normalisation of the experience, and allow practical knowledge to be shared. These interactions constitute a source of vicarious learning and social persuasion that restores the sense of competence and preserves the sense of belonging in the face of isolation produced by chronic pain.	24	70
Restoration of agency through prosociality	Deliberate engagement in behaviours of helping, listening, or transmitting experience to others, enabling the person to experience themselves as competent, useful, and active despite pain. By occupying a supportive or resource-providing role for others, the person produces direct experiences of mastery that counterbalance the sense of uselessness and restore the sense of control over their social place.	12	19
Agentic assertion in the face of social and medical assignments	Deliberate behaviours through which the person resists reductive representations, stereotypes, and injunctions associated with chronic pain, and asserts their needs, limits, and decisions in relation to healthcare professionals, close others, and institutions. This includes the transformation of anger in response to injustice into agentic action. These behaviours restore the sense of control over social identity and the care trajectory, in direct response to threats to autonomy and recognition.	16	33
Regulation of social positioning through distancing	Deliberate strategies of distancing from groups, contexts, or interactions likely to assign the person to an identity centred on pain. This takes three forms: refusal to define oneself through illness, avoidance of interactions focused on complaint, and selective participation without identity commitment. These strategies restore the sense of control over social positioning and preserve an identity not reducible to the pain condition.	16	29

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Category/code	Definition	Interviews	Segments
Emotional regulation in response to pain	Cognitive, attentional, and behavioural strategies through which the person modulates the affective impact of pain to preserve psychological balance. This includes cognitive requalification of pain, such as temporal, comparative, spiritual, or humorous reframing, attentional distraction through regulating activities, and recourse to psychological support. It may include ambivalent behaviours, such as addictions, which are documented without being normalised.	20	39
Strategic regulation of pain visibility	Deliberate management of the social visibility of pain according to a contextual evaluation of the costs and benefits of exposure. The person modulates what they allow others to perceive of their pain, including physical signs, complaints, and medical information, in order to control others' representations and behaviours towards them, preserve their autonomy, and maintain control over their social participation. This takes two complementary forms: active concealment and strategic disclosure.	25	82
Active concealment of pain	Intentional strategies through which the person minimises or masks the expression of their pain, including physical signs, complaints, and medical information, in social or professional interactions, in order to avoid stigmatisation, pity, or disqualification, not to weigh on others, and to preserve a self-image perceived as normal, autonomous, and integrated.	24	69
Strategic disclosure of pain	Voluntary and contextually adjusted disclosure of pain, after evaluation ofthe relational context, when its explicit expression is considered necessary to negotiate adjustments, maintain mutual understanding, or adjust expectationsand roles in the interaction.This restores the sense of control over the conditions of social participation.	9	13
Strategic reorganisation of roles and commitments	Deliberate redistribution of social, professional, and domestic commitments through prioritisation of certain activities and reduction or abandonment of others, in response to constraints imposed by chronic pain. This includes reorganisation of professional roles, restructuring of the care pathway as a priority commitment, and sometimes a deeper revision of values and life priorities. These adjustments preserve the sense of control over social participation despite limitations.	25	56
Category 5 – Embodied experience and cognitive-perceptual modulation of chronic pain	Subjective, embodied, and evolving experience of chronic pain, whose perceived intensity is dynamically modulated by attentional, emotional, and relational states. The social environment constitutes a source of amplification or attenuation of this experience, influencing the sense of bodily control and social sources of self-efficacy. The resistance of this experience to ordinary language leads people to develop cognitive work of symbolisation in order to make it communicable.	30	153
Lived singularity of chronic pain	Deeply subjective and evolving experience of pain, whose perceived intensity, meaning, and effects vary according to personal history, life trajectory, and emotional context. This singularity produces a recurrent gap between objectifiable biological markers and the intensity of lived experience, making pain resistant to standardised measures and partly incommunicable from one individual to another, even with the same diagnosis.	23	48

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Category/code	Definition	Interviews	Segments
Cognitive-affective overload and amplification of pain	Process through which chronic pain saturates attentional and emotional resources, to the point of monopolising the available cognitive space. Negative affective states, such as stress or upset, and pain mutually amplify each other through feedback loops, while attentional engagement in an absorbing activity can temporarily attenuate the pain experience, documenting the bidirectionality of the coupling between attentional allocation and perceptual modulation of pain.	26	94
Metaphorical symbolisation of the pain experience	Use of metaphors, comparisons, and concrete images to figure a pain experience that is diffuse, invisible, or exceeds ordinary language. This cognitive work of shaping enables the person to organise pain perception, articulate its bodily and psychological dimensions, and make it communicable to healthcare professionals, close others, or oneself, where direct description fails.	8	11